

Proposed packaging material		
Code	PENSUS-I-MY-04.01	
Size		
Submission	☐ NDA ☐ Renewal ☒ Variation change detail no.: #270509	
Code of previous version	PENSUS-I-MY-03.01	
Changes	CCDS version 19: Strengthened warning related to nephrotoxicity following signal endorsement	
	☒ CCDS version: 19 ☐ Core PIL version:	☐ SPC country/version/date: ☐ LAC no.:
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PENTASA® Enema 1 g

QUALITATIVE AND QUANTITATIVE COMPOSITION

Each enema contains 1g (1g/100mL) mesalazine.

Excipients: disodium edetate, sodium metabisulphite, sodium acetate, purified water and hydrochloric acid for pH adjustment.

PHARMACEUTICAL FORM

Enema.

APPEARANCE

White to slightly yellow suspension with a pH value between 4.4 and 5.0.

THERAPEUTIC INDICATIONS

Treatment of ulcerative proctosigmoiditis.

POSODOGY AND METHOD OF ADMINISTRATION

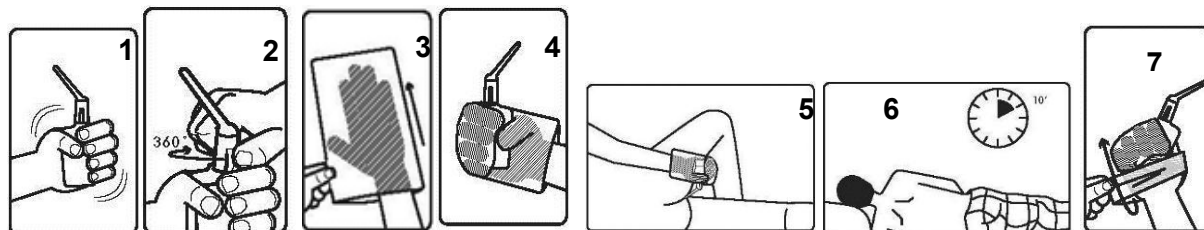
1 enema at bedtime.

Paediatric population: There is little experience and only limited documentation for an effect in children.

PENTASA Enema are for rectal administration. Shake the enema container well before use. The enema is protected by an aluminium foil bag and should be used immediately after opening of the bag.

INSTRUCTIONS FOR USE AND HANDLING

1. A visit to the toilet is recommended before administration of the enema.
2. Immediately before use take the enema bottle out of the aluminium foil pack and shake it well (Figure 1).
3. To break the seal twist the nozzle clockwise one full turn (the nozzle should then be in the same direction as before turning) (Figure 2).
4. Put your hand in one of the plastic disposal bags provided in the pack (Figure 3).
5. Hold the container as shown in the picture (Figure 4).
6. To administer the enema, lie on your left side with the left leg straight and the right leg bent forward for balance. Carefully insert the applicator tip into the rectum. Maintain sufficient steady hand pressure while dispersing the bottle content. The bottle content should be applied within max. 30-40 seconds (Figure 5).
7. Once the bottle is empty, withdraw the tip with the bottle still compressed.
8. The enema should be retained in the bowel. Remain relaxed in the administration position for 5-10 minutes or until the urge to pass the enema has disappeared (Figure 6).
9. Roll the plastic disposal bag over the empty bottle (Figure 7). Discard it and wash your hands.



CONTRAINDICATIONS

Hypersensitivity to mesalazine, any of the excipients, or salicylates.

Severe liver or renal impairment.

SPECIAL WARNINGS AND PRECAUTIONS FOR USE

Most patients who are intolerant or hypersensitive to sulphasalazine are able to take PENTASA® without risk of similar reactions. However, caution is recommended when treating patients allergic to sulphasalazine (risk of allergy to salicylates). Severe cutaneous adverse reactions (SCARs), including Drug reaction with eosinophilia and systemic symptoms (DRESS), Stevens-Johnson syndrome (SJS) and toxic epidermal necrolysis (TEN), have been reported in association with mesalazine treatment. In case of acute intolerance reactions such as abdominal cramps, acute abdominal pain, fever, and severe headache and/or the first appearance of signs and symptoms of severe skin reactions, such as skin rash, mucosal lesions, or any other signs of hypersensitivity, therapy should be discontinued immediately.

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Caution is recommended in patients with impaired liver function. Liver function parameters like Alanine Aminotransferase (ALT) or Aspartate aminotransferase (AST) should be assessed prior to and during treatment, at the discretion of the treating physician.

The drug is not recommended for use in patients with renal impairment. The renal function should be monitored regularly (e.g. serum creatinine), especially during the initial phase of treatment. Urinary status (dip sticks) should be determined prior to and during treatment at the discretion of the treating physician. Mesalazine induced nephrotoxicity should be suspected in patients developing renal dysfunction during treatment. **Mesalazine treatment should be discontinued immediately if renal function deteriorates.** The concurrent use of other known nephrotoxic agents should increase monitoring frequency of renal function.

Patients with pulmonary disease, in particular asthma, should be very carefully monitored during a course of treatment, see **Undesirable effects**.

Mesalazine-induced cardiac hypersensitivity reactions (myo- and pericarditis) have been reported rarely. Serious blood dyscrasias have been reported very rarely with mesalazine. Blood test for differential blood count is recommended prior to and during treatment, at the discretion of the treating physician. As stated in section **Interaction with Other Medicinal Products and Other Forms of Interaction**, concomitant treatment with mesalazine can increase the risk of blood dyscrasia in patients receiving azathioprine, or 6-mercaptopurine or thioguanine. Treatment should be discontinued on suspicion or evidence of these adverse reactions.

Patients with inflammatory bowel disease are at risk of developing nephrolithiasis. Cases of nephrolithiasis have been reported with the use of mesalazine including stones with a 100% mesalazine content. It is recommended to ensure adequate fluid intake during treatment.

As a guideline, follow-up tests are recommended 14 days after commencement of treatment, then a further two to three tests at intervals of 4 weeks. If the findings are normal, follow-up tests should be carried out every three months. If additional symptoms occur, these tests should be performed immediately.

Mesalazine may produce red-brown urine discoloration after contact with sodium hypochlorite bleach (e.g., in toilets cleaned with sodium hypochlorite contained in certain bleaches).

INTERACTION WITH OTHER MEDICINAL PRODUCTS AND OTHER FORMS OF INTERACTION

Combination therapy with PENTASA® and azathioprine, or 6-mercaptopurine or thioguanine have in several studies shown a higher frequency of myelosuppressive effects, and an interaction seems to exist, however, the mechanism behind the interaction is not fully established. Regular monitoring of white blood cells is recommended and dosage regime of thiopurines should be adjusted accordingly.

The concurrent use of mesalazine with other known nephrotoxic agents, such as NSAIDs and azathioprine, may increase the risk of renal reactions.

There is weak evidence that mesalazine might decrease the anticoagulant effect of warfarin.

FERTILITY, PREGNANCY AND LACTATION

PENTASA® should be used with caution during pregnancy and lactation and only if the potential benefits outweigh the possible hazards in the opinion of the physician. The underlying condition itself (Inflammatory bowel disease/IBD) may increase risks for adverse pregnancy outcome.

Pregnancy

Mesalazine is known to cross the placental barrier and its concentration in umbilical cord plasma is lower than the concentration in maternal plasma. The metabolite acetyl-mesalazine is found at similar concentrations in umbilical cord and maternal plasma. There are no adequate and well-controlled studies of PENTASA® use in pregnant women. Limited published human data on mesalazine show no increase in the overall rate of congenital malformations. Some data show an increased rate of preterm birth, stillbirth, and low birth weight; however, these adverse pregnancy outcomes are also associated with active inflammatory bowel disease.

Blood disorders (pancytopenia, leucopenia, thrombocytopenia, anaemia) have been reported in new-borns of mothers being treated with PENTASA®.

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Breastfeeding

Mesalazine is excreted in breast milk. The mesalazine concentration in breast milk is lower than in maternal blood, whereas the metabolite acetyl-mesalazine appears in similar or increased concentrations. No controlled studies with PENTASA® during breast-feeding have been carried out. Hypersensitivity reactions like diarrhoea in the infant cannot be excluded. If the infant develops diarrhoea, breast-feeding should be discontinued.

Fertility

Animal data on mesalazine show no effect on male and female fertility.

EFFECTS ON ABILITY TO DRIVE AND USE MACHINES

Treatment with PENTASA® is unlikely to affect the ability to drive and/or use machines.

UNDESIRABLE EFFECTS

Summary of the safety profile

The most frequent adverse reactions seen in clinical trials are diarrhoea, nausea, abdominal pain, headache, vomiting, and rash.

Hypersensitivity reactions and drug fever may occasionally occur, and severe cutaneous adverse reactions (SCARs), including Drug reaction with eosinophilia and systemic symptoms (DRESS), Stevens-Johnson Syndrome (SJS) and Toxic epidermal necrolysis (TEN), have been reported in association with mesalazine treatment (see section **Special warnings and precautions for use**).

Following rectal administration local reactions such as pruritus, rectal discomfort and urge may occur.

Frequency of adverse effects, based on clinical trials and reports from post-marketing surveillance.

MedDRA Organ Class	Common (≥1/100 to <1/10)	Rare (≥1/10,000 to <1/1,000)	Very rare (<1/10,000)	Not known (cannot be estimated from the available data).
Blood and the lymphatic system disorders			Altered blood counts (anaemia, aplastic anaemia, agranulocytosis, neutropenia, leukopenia (including granulocytopenia), pancytopenia, thrombocytopenia and eosinophilia (as part of an allergic reaction))	
Immune system disorders			Hypersensitivity reaction including anaphylactic reaction	
Nervous system disorders	Headache	Dizziness	Peripheral neuropathy	
Cardiac disorders		Myo*- and pericarditis*		
Respiratory, thoracic and mediastenal disorders			Allergic and fibrotic lung reactions (including dyspnoea, coughing, bronchospasm, allergic alveolitis), pulmonary eosinophilia, interstitial lung disease, pulmonary infiltration, pneumonitis	

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MedDRA Organ Class	Common (≥1/100 to <1/10)	Rare (≥1/10,000 to <1/1,000)	Very rare (<1/10,000)	Not known (cannot be estimated from the available data).
Gastrointestinal disorders	Diarrhoea, abdominal pain, nausea, vomiting, flatulence	Increased amylase, acute pancreatitis*	Pancolitis	
Hepato-biliary disorders			Increase in transaminases, increase in cholestasis parameters (e.g. alkaline phosphatase, gamma-glutamyltransferase and bilirubin), hepatotoxicity (incl. hepatitis*, cholestatic hepatitis, cirrhosis, hepatic failure)	
Skin and subcutaneous tissue disorders	Rash (incl. urticaria, erythematous rash)	Photosensitivity**	Alopecia Reversible, dermatitis allergic, erythema multiforme	Stevens-Johnson Syndrome (SJS)/ Toxic epidermal necrolysis (TEN), Drug Reaction with Eosinophilia and Systemic Symptoms (DRESS)
Musculoskeletal, connective tissue and bone disorders			Myalgia, arthralgia, lupus erythematosus-like syndrome (systemic lupus erythematosus)	
Renal and urinary disorders			Renal function impairment (incl. acute and chronic interstitial nephritis*, nephrotic syndrome, renal insufficiency)	Nephrolithiasis*** Urine discolouration***
Reproductive system disorders			Oligospermia (reversible)	
General disorders and administration site conditions	Anal discomfort and irritation at the application site, pruritus (anal), rectal tenesmus		Drug fever	

(*) The mechanism of mesalazine-induced myo- and pericarditis, pancreatitis, nephritis and hepatitis is unknown, but it might be of allergic origin.

(**) Photosensitivity: More severe reactions are reported in patients with pre-existing skin conditions such as atopic dermatitis and atopic eczema.

(***) See section **Special warnings and precautions for use** for further information.

It is important to note that several of these disorders can also be attributed to the inflammatory bowel disease itself.

OVERDOSE

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There is limited clinical experience with overdose of PENTASA® which does not indicate renal or hepatic toxicity. Since PENTASA® is an amino salicylate, symptoms of salicylate toxicity, such as acid-base balance disorder, hyperventilation, pulmonary edema, vomiting, dehydration and hypoglycaemia may occur. Symptoms of salicylate over dosage are well described in the literature.

There have been reports of patients taking daily doses of 8 grams for a month without any adverse events.

There is no specific antidote and the management of overdose is supportive and symptomatic. The treatment at the hospital includes close monitoring of renal function.

PHARMACODYNAMIC PROPERTIES

Pharmacotherapeutic group: Intestinal anti-inflammatory agents (A07 EC02)

Mechanism of action and pharmacodynamic effects: It has been established that mesalazine is the active component of sulfasalazine, which is used for the treatment of ulcerative colitis and Crohn's disease. Based on clinical results, the therapeutic value of mesalazine after oral as well as rectal administration appears to be due to local effect on the inflamed intestinal tissue, rather than to systemic effect. There is information suggesting that severity of colonic inflammation in ulcerative colitis patients treated with mesalazine is inversely correlated with mucosal concentrations of mesalazine.

Increased leucocyte migration, abnormal cytokine production, increased production of arachidonic acid metabolites, particularly leukotriene B4, and increased free radical formation in the inflamed intestinal tissue are all present in patients with Inflammatory Bowel Disease (IBD). The mechanism of action of mesalazine is not fully understood although mechanisms such as activation of the γ -form of peroxisome proliferator-activated receptors (PPAR- γ) and inhibition of nuclear factor-kappa B (NF- κ B) in the intestinal mucosa have been implicated. Mesalazine has in-vitro and in-vivo pharmacological effects that inhibit leucocyte chemotaxis, decrease cytokine and leukotriene production, and scavenge for free radicals. It is currently unknown which, if any, of these mechanisms play a predominant role in the clinical efficacy of mesalazine.

PHARMACOKINETIC PROPERTIES

General characteristics of the active substance

Disposition and local availability:

The therapeutic activity of mesalazine most likely depends on a local contact of the drug with the diseased area of the intestinal mucosa.

PENTASA® enemas are designed to provide the distal part of the intestinal tract with high concentrations of mesalazine and a low systemic absorption. Enemas have been shown to reach and cover the descending colon.

Absorption:

The absorption following rectal administration is low, and depends on the dose, the formulation and the extent of spread. Based on urine recoveries in healthy volunteers under steady-state conditions given a daily dose of 2g (1g x 2), about 15-20% is absorbed after administration of enemas.

Distribution:

Protein binding of mesalazine is approximately 50% and of acetyl-mesalazine about 80%.

Elimination:

Once the formulation is not present in the gastrointestinal tract elimination will follow the plasma half-life of orally or IV administered uncoated mesalazine, which is approximately 40 minutes and for acetyl-mesalazine approximately 70 minutes.

The systemic exposure following administration of PENTASA® enemas has been shown to be significantly decreased in patients with active ulcerative colitis as compared to those in remission.

INCOMPATIBILITIES

None known.

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SHELF LIFE

2 years.

SPECIAL PRECAUTIONS FOR STORAGE

Store below 30°C. Do not refrigerate or freeze. Store in the original package, as the product is sensitive to light.

PACK SIZES

Polyethylene bottles with a tip for rectal application. The bottles are supplied in aluminium foil bags. Box of 7 bottles and 7 PE bags.

SPECIAL PRECAUTIONS FOR DISPOSAL

The enema may colour the linen and toilet.

MANUFACTURER

Ferring-Léčiva, a.s.,
K Rybníku 475, 252 42 Jesenice u Prahy, Czech Republic

Date of revision: October 2025